

# DECAN TABLET

## **Ingredient(s):**

Each tablet contains:

Dexamethasone ..... 0.75mg

## **Pharmacology (Summary of Pharmacodynamics and Pharmacokinetics):**

### **Pharmacodynamics:**

1. Dexamethasone is a synthetic glucocorticoid and has potent anti-inflammatory properties. It is indicated for the treatment of endocrine disorders, rheumatic disorders, collagen diseases, dermatologic diseases, allergic states and etc.
2. Dexamethasone can decrease or prevent tissue responses to inflammatory processes, thereby reducing development of symptoms of inflammation without affecting the underlying cause. Dexamethasone inhibits accumulation of inflammatory cells, including macrophages and leukocytes, at sites of inflammation. It also inhibits phagocytosis, lysosomal enzyme release, and synthesis and/or release of several chemical mediators of inflammation.

### **Pharmacokinetics:**

Dexamethasone is readily absorbed from the gastrointestinal tract. Its biological half-life in plasma is about 190 minutes. Binding of Dexamethasone to plasma proteins is less than most of the other corticosteroids.

### **Indication(s):**

Acute rheumatism, rheumatoid arthritis, acute gouty arthritis, bronchial asthma, serum sickness, diffusum lupus erythematosus, nodular arthritis, allergic dermatitis, pemphigus, bursitis, myositis, fibromyositis, and inflammatory ophthalmic diseases.

### **Dosage and Administration:**

Usual adult dose: 0.75mg, 2 to 4 times a day.

To be taken after meals and at bedtime or in the morning and at night.

The daily dose may be reduced by 0.25 - 0.5mg for every 4 - 5 days interval until the minimum maintenance dose is attained or all the symptoms have completely disappeared.

### **Route of Administration:**

For oral administration only.

### **Contraindication(s):**

Dexamethasone is contraindicated in patients with systemic fungal infections and those who have shown hypersensitivity to any component of this product.

### **Precaution(s) / Warning(s):**

1. The adverse effects of Dexamethasone are almost always due to its use in excess of normal physiological requirements. They should be treated symptomatically, where possible the dosage is reduced or the drug slowly withdrawn.
2. Dexamethasone may mask some signs of infections and new infections that may appear during its use. There may be decreased resistance and inability to localise infections when Dexamethasone is used.
3. Dexamethasone should be used cautiously in patients with ocular herpes simplex, psychic derangements and gastric-duodenal ulcer.
4. Unless considered life-saving, systemic administration of corticosteroids is contraindicated in patients with osteoporosis, psychoses, or severe psychoneurosis, and they should be used only with great caution in the presence of congestive heart failure or hypertension, in patients with diabetes mellitus, chronic renal failure, and uremia, and in elderly person.
5. Pheochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified pheochromocytoma after an appropriate risk/benefit evaluation.

### **Interactions with Other Medicament(s):**

Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side effects, in which case patients should be monitored for systemic corticosteroid side effects.

### **Pregnancy and Lactation:**

The use of Dexamethasone in pregnancy, nursing mothers, or women of childbearing potential requires that a possible benefits of the drug be weighed against the potential hazards, to the mother and embryo fetus.

### **Side Effect(s) / Adverse Reaction(s):**

1. The prolonged therapy may result in suppression of pituitary-adrenal function and the principal complications resulting from prolonged therapy with Dexamethasone are fluid and electrolyte disturbances; hyperglycemia and glycosuria; increased susceptibility to infections including tuberculosis; peptic ulcers, which may bleed or perforate; osteoporosis; a characteristic myopathy; behavioral disturbances; posterior subcapsular cataracts; arrest of growth; and Cushing's habitus, consisting of "moon face", "buffalo hump", enlargement of supraclavicular fat pads, "central obesity", striae, ecchymoses, acne, and hirsutism.

2. Acute adrenal insufficiency results from too rapid withdrawal of corticosteroids after prolonged therapy. Dexamethasone withdrawal syndrome, consisting of fever, myalgia, arthralgia and malaise.

**Symptoms and Treatment for Overdosage, and Antidote(s):**

There is no specific antidote for its overdose, therefore, management of the patient consists of symptomatic and supportive therapy.

**Shelf-Life:**

3 years from the date of manufacture.

**Storage Condition(s):**

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

**Product description:**

A white color round tablet, one side impressed with a letter “S”. 

**Packing(s):**

Plastic bottle of 1000's and 2000's (for export only).

Blister packing of 10's x 10 and 10's x 100

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**Bahan Aktif:**

Setiap tablet mengandung:

Dexamethasone ..... 0.75mg

**Indikasi:**

Reumatisme akut, reumatoid arthritis, akut gout arthritis, asma, serum sickness, diffusum lupus eritematosus, arthritis nodul, alergi dermatitis, pemfigus, bursitis, miositis, fiber miositis dan penyakit inflamasi oftalmik.

**Dos:**

Dewasa: 0.75mg, 2 - 4 kali sehari, diambil selepas makan dan sebelum tidur atau pada pagi dan malam. Kurangkan dos beransur-ansur sebanyak 0.25 - 0.5mg setiap 4 - 5 hari sehingga dos maintenans minima dicapai atau kesemua simptom dapat dikawal.



Manufacturer and Product Registration Holder:  
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